EMERGENCY NEUROLOGY CONSULTATION: Acute Stroke Code Activation.

PATIENT: A 68-year-old right-handed female with atrial fibrillation and hypertension who was last seen normal by her spouse at 08:30. At 09:15, she developed acute-onset inability to speak and profound right-sided weakness. Arrival to the Emergency Department was at 09:40 (symptom onset window 70 minutes).

NEUROLOGICAL EXAMINATION (BASELINE NIH STROKE SCALE):

Level of Consciousness: Alert, oriented to person only (1 point). Answers month and age incorrectly (2 points). Follows commands (0 points).

Best Gaze: Partial right gaze palsy that can be overcome by doll's-head maneuver (1 point).

Visual Fields: Complete right homonymous hemianopia by confrontation (2 points).

Facial Palsy: Right central lower facial flattening with asymmetry on smiling (2 points).

Motor Arm: Right arm drifts down to stretcher immediately without effort against gravity (4 points); left arm normal (0 points).

Motor Leg: Right leg falls to bed in less than 5 seconds (3 points); left leg normal (0 points).

Limb Ataxia: Absent bilaterally in non-paretic limbs (0 points).

Sensory: Severe loss of pinprick and light touch sensation over right face, arm, and leg (2 points).

Best Language: Global aphasia; completely non-fluent with severe paraphasic errors, inability to name common objects or repeat phrases (3 points).

Dysarthria: Severe slurring on attempted vocalization (2 points).

Extinction and Inattention: Profound visual and tactile hemi-inattention to right-sided sensory stimuli (2 points).

Total Baseline NIHSS Score: 24 (Severe Stroke).

NEUROIMAGING:

Emergency Non-contrast Head CT: Completed at 09:55. Alberta Stroke Program Early CT Score (ASPECTS) is 9/10 with subtle loss of the left insular ribbon. No evidence of acute intracranial hemorrhage, mass effect, or midline shift.

CT Angiography (CTA) Head and Neck: Demonstrates an abrupt cutoff and occlusive thrombus in the left middle cerebral artery at the M1 segment just distal to the bifurcation. Good collateral reconstitution via leptomeningeal vessels.

MANAGEMENT:

1. Patient meets all clinical and radiographic criteria for intravenous thrombolysis within the 4.5-hour therapeutic window. Blood pressure confirmed at 155/90 mmHg, blood glucose 118 mg/dL, platelets 210,000/uL, INR 1.1 (patient was not taking prescribed anticoagulants).

2. Administered intravenous tenecteplase (TNKase) at 0.25 mg/kg (total 17.5 mg IV bolus over 5 seconds) at 10:05.

3. Emergent transfer to the neurointerventional catheterization laboratory for mechanical thrombectomy of the left MCA M1 occlusion.